CODE 6 — ON HEALTH

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PRELIMINARY TITLE — FUNDAMENTAL PRINCIPLES

Art. 1 — Subject matter and legal nature

1. This Code governs the protection of health as an infrastructural commons, an inviolable right and a material condition of existence of the Confederation, in accordance with Article 1.2 of the Constitution.
2. Health is not a commodity, a paid service or a privilege linked to the possession of competences or to the amount of CBO performed.
3. For the purposes of this Code, health means the physical, mental, relational and environmental well-being of the person. Its assessment respects individual self-determination and takes into account the collective conditions that affect health.
4. This Code governs:
    a) the prevention of risks connected with the CBO, the Storehouses and habitats;
    b) health surveillance and coordination with the safety of tasks;
    c) primary care, the care of frail persons and mental health;
    d) the management of accidents and occupational diseases;
    e) the Health Basket and pharmaceutical autonomy;
    f) the confidentiality of health data.
5. The competences under this Code are coordinated with those under Codes 12 and 14. The Health Council does not replace the bodies competent for general safety, the care of frail persons or data protection.

Art. 2 — Principles

1. **Prevention.** CBO, production, logistics and learning activities are organised so as to prevent harm to health before it occurs.
2. **Self-determination.** Every person decides on their own body and on their own course of care on the basis of free, informed and revocable consent. Article 2-ter applies to minors and to those who are unable to decide. The collective measures of Article 2-bis may limit tasks, activities and movements within the limits laid down therein, but never include an imposed health treatment. Article 8 remains applicable to a person temporarily incapable in acute crisis.
3. **No blame.** Illness, accident and frailty do not constitute fault and may not, in themselves, entail a reduction of the Sufficiency Basket, exclusion or sanction. The guarantees of Code 4 apply.
4. **Proximity.** Care is organised, as far as possible, in the person’s Circle, Storehouse and habitat. Recourse to hospital or centralised services is ordered when it is necessary in order to access Level 3 competences not available locally.
5. **Non-commodification of knowledge.** It is prohibited to patent life-saving knowledge and essential health knowledge or to place them behind a paywall. Rent deriving from the monopoly of health competences is prohibited, in accordance with Code 5, Article 8.
6. **Proportionality and confidentiality.** Every health measure is limited to the purpose for which it is adopted. Health information is processed only to the extent necessary and with the guarantees of Article

11.

Art. 2-bis — Immediate collective risk and coordination with Article 8

1. **Immediate and documented collective risk** means a situation in which a pathogen, a contaminant or an environmental condition, on the basis of verifiable scientific evidence recorded in the Health Knowledge Storehouse, entails a concrete, present and serious danger of spread or of harm to third parties that cannot be avoided by less invasive measures.

2. The assessment is initiated on a proposal from the Health Council and requires:

   a) a dossier describing the evidence, the risk, the persons or groups potentially exposed and the alternatives considered;

   b) the opinion of at least two persons with Level 3 competence, who are independent and do not belong to the proposing Circle;

   c) validation of the applicable STV, where provided for.

3. Collective measures are resolved upon by the General Assembly in accordance with the urgency procedure of Code 3. The resolution shall state its reasons and document necessity, proportionality, duration, less invasive alternatives and criteria for cessation. Coordination with Code 14 applies where matters of general safety or of interruption of activities are involved.

3-bis. The collective measures may comprise exclusively:

a) public information, recommendations and the free offer of tests, vaccinations, prophylaxis and PPE, which remain voluntary;

b) the protection indicated by the resolution, from among vaccination, prophylaxis, tests and PPE, as a condition for performing the tasks and for attending the places at risk, on the conditions of paragraph 3-ter;

c) the temporary suspension or reorganisation of collective activities, meetings, CBO shifts, non-essential mobility and non-urgent hospitality;

d) the isolation of contagious persons and the quarantine of exposed persons, on the conditions of paragraph 3-quater;

e) measures concerning contaminated environments, water, food and goods and, for zoonoses, the measures concerning animals permitted by Code 13.

Only the tasks and places in which contagion may affect particularly exposed persons are at risk: wards and clinics, the care of frail persons pursuant to Code 12 and, where the risk concerns childhood, spaces for childhood and places of collective learning. No one may be excluded from the care they need or from assisting a hospitalised person for whose care they are responsible; in these cases alternative precautions are adopted.

3-ter. No measure includes a health treatment, a sample, a test or a vaccination performed on the body of a person without the consent of that person or, for those who cannot decide for themselves, of the person who decides pursuant to Article 2-ter. Whoever refuses the protection indicated pursuant to paragraph 3-bis, letter b):

a) may be excluded, for the duration of the measure, only from the tasks and places at risk;

b) shall receive without delay another compatible task, counted in the CBO; if none is available, the temporary suspension of Article 11, paragraph 1, letter c), of Code 1 applies. A minor excluded from a collective space shall be ensured care and learning in another form;

c) retains the Basket, housing, care, the vote, deliberative participation and the other rights, including during the measure;

d) does not commit an offence; the refusal is not recorded or communicated beyond what is necessary to apply letter a).

3-quater. Isolation and quarantine:

a) take place in the person's dwelling or, if the person prefers or the dwelling does not allow it, in a suitable space made available by Circle 1;

b) last as long as the dossier of paragraph 2 indicates for the agent in question and in any case no more than fourteen days per person and per exposure; one extension, for no more than a further fourteen days, is permitted only if two persons with L3 competence certify that the person is still contagious or, for quarantine, that the incubation period indicated by the dossier has not ended. Beyond twenty-eight days the isolation of a person who is still contagious and does not consent to it may be ordered only by the Guarantee Tribunal, after hearing the person with the guarantees of Article 25, paragraph 4, of Code 4, on the certification of two independent L3 persons pursuant to paragraph 2, letter b), for periods not exceeding fourteen days and not beyond the duration of the collective measure; failing that, only paragraph 3-ter applies;

c) if the person does not consent, are ordered individually by the Health Council by reasoned act, communicated without delay to the Guarantee Tribunal, which shall validate, modify or revoke them within twenty-four hours pursuant to Article 34, paragraph 2, of Code 4; in the absence of validation they cease. The same validation applies to the extension. Paragraph 3 of that Article does not apply;

d) ensure for the person the delivery of the Basket, care, remote contact with loved ones, voting through the Network and the temporary suspension of the CBO provided for by Article 11, paragraph 1, letter c), of Code 1, except for the tasks that the person asks to perform remotely;

e) are not enforced by force, except for the intervention of the Civic Protection Corps within the limits of Article 8-ter of Code 14; the validation of letter c) also applies for the purposes of paragraph 4 of that Article;

f) during the transition, if the person does not consent, are ordered only by the health authorities of the State, pursuant to Article 37 of Code 4.

3-quinquies. For the entire duration of the measure the proposing Health Council shall publish, at least every week, the dossier of paragraph 2, the aggregated and anonymous data on which the measure is based, their trend and the reasons for each renewal or cessation, in compliance with Article 11.

4. The measure:

   a) lasts no more than thirty days;

   b) may be renewed only by a new resolution and with new documentation;

   c) is limited to what is strictly necessary and adopts the least invasive suitable measure;

   d) is reviewed, at the request of any member, within seven days, by the members of the Guarantee Circles drawn by lot pursuant to Article 17, paragraph 1-bis, of Code 3; individually ordered isolation and quarantine instead follow the validation of paragraph 3-quater, letter c);

   e) is recorded with its reasons, duration and criteria for cessation, in compliance with confidentiality and with the access levels provided for by the Codes.

5. When the conditions for it cease to exist, the measure ceases immediately. Renewal may not be based on mere inertia or on the fact that a review has not been concluded.

6. This Article governs public health measures addressed to the community as a whole. Article 8 governs individual health emergency interventions concerning a person in crisis. The two regimes have distinct subject matter, conditions and procedures and are not cumulative, unless both sets of conditions are documented.

7. Measures adopted pursuant to this Article may not reduce, not even while they apply, the Basket, housing, care, the vote and deliberative participation, nor produce permanent effects on access to the CBO or to the other rights. Any further effects require a separate procedure provided for by the competent Codes, together with the related guarantees.

8. When the measure ceases, the person's previous legal position is restored. The records necessary to

document, verify or audit the measure may be retained, with restricted access and in accordance with the confidentiality guarantees of Article 11.

Art. 2-ter — Consent of minors and of persons who cannot decide

1. From completion of the sixteenth year the person gives, refuses and revokes consent to their own care themselves, pursuant to Article 2, paragraph 2. If, before the eighteenth year, the person is unable to decide, paragraph 2 applies.

2. Until completion of the sixteenth year consent is given by those who hold responsibility for the care of the minor pursuant to Article 24.3 of the Constitution, after informing and hearing the minor in a manner suited to his or her age. The opinion of the minor carries increasing weight with age and maturity, pursuant to Article 26.2 of the Constitution. If there is more than one responsible person, they decide by agreement; for urgent care the consent of one of them suffices. Responsibility for care and the consent of this paragraph do not cease by reason only of the cessation of the member status of the responsible person pursuant to Article 29, paragraph 6-bis, of Code 4. If no one exercises responsibility for care, or no responsible person can be reached in good time, consent is given by the temporary representative appointed pursuant to Article 7-bis, paragraph 5, of Code 12, until the decision of the Guarantee Tribunal provided for by paragraph 3-bis of that Article.

3. In the event of disagreement between the minor and those who hold responsibility for his or her care, or between several responsible persons, or of a conflict of interest between them and the minor, the minor, each responsible person, the treating practitioner or the Care Council may apply to the Guarantee Tribunal. The Tribunal obtains the written opinion of a person with L3 health competence, independent and not involved in the care, drawn by lot from the list of Article 36, paragraph 1, letter b), of Code 4; it hears the minor, assisted by the trusted person provided for by Article 31.5 of the Constitution, and the responsible persons; it decides within seven days or, if the care cannot wait, within twenty-four hours, according to the interest of the minor and taking account of his or her opinion pursuant to paragraph 2. Where waiting would expose the minor to a danger to life or to serious and irreversible harm, the necessary care is provided immediately, pursuant to Article 28.2 of the Constitution, and the Tribunal is informed.

4. Vaccinations and prophylaxis of minors follow paragraphs 1 to 3 and are never mandatory. Refusal of a vaccination does not, by itself, constitute serious neglect or harm within the meaning of Article 7-bis of Code 12. During a measure under Article 2-bis a minor who does not have the protection indicated may be excluded only from the places at risk, on the conditions of paragraph 3-ter of that Article.

5. Where an adult person is unable to decide on their own care because of a condition that lasts, or is expected to last, more than sixty days, as established by two independent persons with L3 health competence, the Guarantee Tribunal shall appoint a stable representative, if the person has not designated one pursuant to Article 7-bis, paragraph 4, of Code 12. The representative is chosen in the order of Article 7-bis, paragraph 5, of Code 12 and decides in accordance with paragraph 6 of that Article. The person is informed and heard to the extent that they can express themselves. The appointment is reviewed by the Tribunal at least every twelve months and at any time at the request of the person, the trusted person, the representative or the Care Council; it ceases when the person recovers capacity. Paragraph 3 applies to disagreement or conflict of interest between the representative and the person. The representative may not consent to coercive measures, which are governed solely by Article 8.

6. During the transition this Article applies within the limits permitted by the law in force on the consent of minors and on the representation of incapable persons.

CHAPTER I — ORGANISATION, SAFETY SHEETS AND PREVENTION

Art. 3 — Health Council

1. Every Circle 1 shall establish a Health Council composed of three to seven members, with Level 2 or 3

competences in the fields of health, care, agroecology or occupational safety. It is the local branch of the Medicine and Health Competence Circle. Its members are drawn by lot in accordance with Article 12-bis of Code 3, remain in office for one year and may not be drawn by lot again for the same Council until twelve months have passed since the end of their term of office.

2. The Health Council:

   a) manages the Health Storehouse and the Health Basket referred to in Article 10;

   b) draws up and updates, as regards the health aspects within its competence, the Safety Sheets for CBO tasks;

   c) organises health surveillance, the management of PPE within the health remit and L1 training on health;

   d) records and analyses accidents, near misses and occupational diseases;

   e) coordinates the care of frail persons and mental health interventions in accordance with Code 12;

   f) coordinates with the other Circles 1 the management of scarce competences and of vital emergencies, in accordance with Article 32 of Code 1 and with Code 14.

3. The Health Council has no allocative powers over goods and no disciplinary powers. It may not deny access to the Basket or make it subject to conditions.

4. The Council formulates proposals, documents health risks, organises assistance and performs the functions assigned to it by this Code. Its decisions are adopted by the consensus method provided for by Code 3 and are tracked.

5. For every CBO task a safety reference person with L2 competence, validated in accordance with Code 5, shall be identified. The reference person does not replace the Health Council or the Safety Council provided for by Code 14.

Art. 4 — Safety Sheets, PPE and coordination with Code 2

1. No CBO activity may be carried out without a Safety Sheet in force, prepared in accordance with the applicable STVs and coordinated with Codes 1 and 14.

2. Where a task has both health aspects and general safety aspects, the Sheet may be a single integrated document or may consist of linked documents bearing the same identifier and the same version. The Health Council validates the health aspects; the Safety Council validates the general safety aspects; the Storehouse Council validates the organisational aspects within its competence. No Council may modify the parameters assigned to the others.

3. The Sheet shall contain at least:

   a) the specific risks;

   b) the PPE and collective protections required;

   c) the operating procedures;

   d) the typical errors and the measures to prevent them;

   e) the conditions for suspension;

   f) the references to the applicable STVs;

   g) the exposure limits and the breaks required;

   h) the minimum training required;

   i) the date, the version, the persons responsible and the conditions for revision.

4. PPE items are commons and are provided free of charge. They are accessible, kept in working order and tracked by those competent for custody and procurement. The absence of a critical item of PPE required by the Sheet entails the immediate suspension of the activity concerned and does not constitute a violation attributable to the person to whom the PPE was not provided. The obligation to make PPE available lies with the Circle responsible for the activity.

5. Every member may stop an activity when they detect a serious and immediate risk to themselves or to

others. The stoppage is communicated as soon as possible to the operational lead and to the competent Council; it may not give rise to retaliation if carried out in good faith. Code 14 applies and, for logistical continuity, Article 32, paragraph 3, of Code 1.

6. The STV of a good includes the health safety aspects relating to production, use, maintenance and disposal. The STV is coordinated with the Safety Sheet and does not replace it where the latter is required for a specific task.

Art. 5 — Health surveillance, voluntary nature and safety conditions

1. Health surveillance is voluntary as a general course of care and prevention. For the tasks identified pursuant to paragraph 2, a fitness assessment may constitute a technical requirement for performing that specific task autonomously; it does not constitute a general health obligation.

2. The following rules apply:

   a) for CBO activities with no risk or low risk, no mandatory examination is required;

   b) for medium-risk or high-risk activities, classified in accordance with the STVs and the Safety Sheet, the fitness assessment is required only when it is necessary for the safety of the autonomous task;

   c) the fitness assessment is not a condition for accessing the Basket, taking part in deliberations or belonging to the Community;

   d) a person who refuses the examination necessary for a medium-risk or high-risk task shall not perform that specific task autonomously, but retains the Sufficiency Basket, access to the other compatible CBO activities and the right to an adapted alternative task. The refusal does not constitute an infringement and is not recorded as a prejudicial annotation;

   e) the periodic examination provided for a high-risk L3 health competence may be a condition for maintaining the related validation, in accordance with Article 4, paragraph 5, of Code 5. Refusal entails, if necessary, the suspension of the specific validation, not a sanction.

3. Health assessments may include:

   a) an initial examination for medium-risk or high-risk tasks, when necessary pursuant to paragraph 2;

   b) a periodic examination for L3 competences, at least every two years, or every year in the case of continuous exposure, in accordance with the applicable STVs;

   c) an examination requested by the person at any time;

   d) an examination following an accident or an occupational disease.

4. The finding is descriptive and limited to the task concerned. It may indicate:

   a) fitness;

   b) fitness with adaptations;

   c) fitness with temporary limitations;

   d) temporary precautionary suspension from the task, with the Basket maintained and an alternative CBO proposed.

5. Definitive unfitness for a specific task may be declared exclusively by an external Review Panel in accordance with the guarantees of Code 4, Article 17. The Panel is composed of:

   a) one person with L3 health competence, drawn by lot and not belonging to the Circle of the person concerned;

   b) one person from the Quality Council not involved in the proceedings;

   c) one person drawn by lot from among the members of the Assembly of a Circle 1 other than the one concerned, with the function of procedural guarantee.

6. Before the decision, the Panel shall obtain the written opinion of two independent persons with L3 health competence, not involved in the previous findings and, where relevant, with different specialisations.

The persons giving the opinion are not members of the Panel and do not take part in the decision.

7. No member of the Panel may have given a previous fitness finding on the same person and task. The opinions are made available to the person concerned before the decision, except for the limitations strictly necessary to protect the health data of third parties.

8. The decision shall state its reasons and be tracked. An appeal against definitive unfitness may be lodged within fourteen days with a new external Panel, composed of different persons. The appeal is admissible on grounds of:

    a) procedural defect;

    b) manifest technical unreasonableness;

    c) discrimination;

    d) manifest disproportion.

Code 4, Article 17, and Code 5, Article 9, apply. The second-instance Panel may not worsen the position of the appellant.

9. During the procedure for establishing definitive unfitness, the person retains the Sufficiency Basket and receives a proposal for a compatible alternative CBO.

10. No health data may be used to limit access to deliberations, to the Basket or to learning.

CHAPTER II — CARE, FRAILTY AND MENTAL HEALTH

Art. 6 — Primary care and Health Storehouse

1. Every Storehouse has a Health Storehouse and a minimum Health Basket. The list of the Health Basket and the minimum stock of the Health Storehouse are proposed by the Health Councils in federated coordination, as branches of the Medicine and Health Competence Circle, by means of an STV adopted pursuant to Article 3-bis of Code 2, and are approved by the Assembly with the Technical Annex provided for by Article 18, paragraph 2, of Code 1. The Basket includes at least:

    a) essential life-saving medicines;

    b) first-aid supplies;

    c) PPE within the health remit;

    d) basic diagnostic instruments;

    e) validated health documentation and STVs.

2. Primary care is organised through mentoring and Circle clinics, with the involvement of L2 and L3 competences. The person chooses among the competences available, including those belonging to other Circles.

3. The AI provided for by Code 7 may support the indexing of protocols, the search for information and reminders. It may not autonomously formulate diagnoses, determine fitness or decide on treatment.

4. When a competence is not available locally, the Health Council activates the network among Circles and the Dependency Register provided for by Code 15. The possible measures include:

    a) sending the person to the competent service;

    b) temporarily sending the competence to the Circle concerned;

    c) remote mentoring supervised by an L3 person, when appropriate and safe.

5. The arrangement chosen shall respect the person’s wishes, health confidentiality and continuity of care.

6. The protection of health is structured in the three integrated levels of Article 28.1 of the Constitution: the offline personal AI with a preventive function, in accordance with Article 5-bis of Code 7; the care practitioners of the Base Community; the centres of high specialisation of Circle 3.

Art. 7 — Frailty, interdependence and care

1. Older persons, children, persons with disabilities, persons with chronic illnesses, persons who are pregnant or in the period following childbirth, and persons in mourning or in a situation of crisis are entitled to enhanced protection and, when necessary, to the modulation of the CBO pursuant to Article 11 of Code 1 and to Code 12.
2. The care of frail persons falls within the List A CBO. In the hierarchy of Article 32 of Code 1 the services of the Care Basket of levels 1 and 2 follow letter b) of paragraph 1 and Code 12; the other care activities follow letter d) and precede the management of perishable goods, pursuant to paragraph 1-ter of that Article.
3. Care activities may not be deferred on account of logistical shortage or of the management of perishable goods, unless the priorities of Article 32, paragraph 1, letters a) and b), of Code 1 so require, including the distribution of Basket food. The decision to defer shall state its reasons and must provide, where possible, for a substitute measure.
4. Every CBO Plan includes a share of care and mentoring. Care is not regarded as a residual activity to be carried out only after the other requirements have been met.
5. The modulation of the CBO does not entail a reduction of the Sufficiency Basket or prejudicial annotations.

Art. 8 — Mental health and protocol for acute crisis

1. Mental health is an integral part of health. Distress, burnout, conflict and trauma do not constitute fault or individual weakness.
2. The Health Council guarantees, directly or through coordination with Code 12:
    a) listening spaces;
    b) talking groups;
    c) L2 or L3 psychological support;
    d) temporary reduction of the CBO load, when necessary;
    e) peer support;
    f) access to specialist competences without an unjustified internal waiting list.
3. In the event of an acute crisis, a non-coercive protection protocol applies as a rule, based on:
    a) presence and listening;
    b) relational and environmental making safe;
    c) an offer of support;
    d) the involvement, if the person so wishes, of a trusted person designated by them.
4. An immediate coercive measure is permitted only if the following conditions are all met cumulatively:
    a) there is an immediate and concrete vital risk to the person or to third parties, documented at the time by two persons with L3 competence, at least one of them in health;
    b) the person is temporarily incapable of giving valid consent;
    c) there is, in the immediate term, no less invasive measure that is equally effective.
5. Where the two L3 competences are not immediately available, the procedures for federated sourcing or remote mentoring provided for by Article 13 of Code 15 are activated without delay. In the meantime, only non-coercive measures necessary for immediate making safe are permitted.
6. Any coercive intervention that is adopted:
    a) is limited to the time strictly necessary to avert the vital risk;
    b) is carried out in the presence of two L3 persons, when immediately available;
    c) is communicated without delay to the Guarantee Tribunal for the validation of paragraph 7 and to

the Guarantee Circle;

d) is documented on the Network with regulated access;

e) is communicated to the designated trusted person, when possible;

f) ceases immediately when the conditions for it cease to exist.

7. Within twenty-four hours of the start of the measure the Guarantee Tribunal, pursuant to Article 27, paragraph 3, of Code 4, shall validate, modify or revoke it. It verifies the conditions of paragraph 4, necessity and proportionality on the basis of the documentation of the two L3 persons and, where possible, after hearing the person, their representative and the trusted person. It does not carry out a clinical assessment, which remains the responsibility of the L3 health persons. In the absence of validation within the time limit the measure ceases. If the Civic Protection Corps intervenes, the validation of this paragraph takes the place of that provided for by Article 8-ter, paragraph 4, of Code 14.

8. After the crisis, a voluntary course of care is offered to the person.

9. Punitive restraint, isolation used as a sanction and punitive sedation are always prohibited.

10. The coercive measure:

    a) is reassessed by the two L3 persons at least every twelve hours and, during physical restraint, at least every hour; every reassessment is documented;

    b) after validation lasts no more than seventy-two hours and may be extended only by a new validation of the Guarantee Tribunal, requested before expiry on the basis of a new certification by two L3 persons, for periods not exceeding seventy-two hours; in the absence of a new validation before expiry the measure ceases;

    c) may not exceed fourteen days in total. On expiry the measure ceases even if the risk persists; the continuous presence of trained persons, the relational and environmental making safe of paragraph 3 and the offer of care are ensured without interruption, and the person may remain voluntarily in the place of care. A new coercive measure is permitted only for a new acute episode, on the conditions of paragraph 4 certified by two L3 persons other than those who certified the previous measure, and with the validation of a Guarantee Tribunal in a different composition, which expressly verifies that it is not the continuation of the measure that has ceased. Within a period of ninety days coercive measures may not exceed twenty-eight days in total;

    d) is revoked at any time by the Guarantee Tribunal, when the conditions for it cease to exist, at the request of the person, their representative or the trusted person; the request is decided within twenty-four hours.

The representative may not consent to a coercive measure in place of the person: only this Article applies. During the transition no internal coercive measures are carried out; every intervention is limited to what is permitted by the law in force and the emergency services and the competent health authorities are alerted, pursuant to Article 37 of Code 4.

11. It is prohibited to insert stigmatising labels in the Personal Portfolio.

Art. 8-bis — Living will and end of life

1. The living will is binding, pursuant to Article 27.3 of the Constitution. It is drawn up by the person, with the assistance, if any, of their own offline personal AI and in the presence of two witnesses; it is kept in the personal Node and, if the person so wishes, with the Health Council in confidential form. It may be revoked and amended at any time.

2. By the living will the person may designate their representative in the event of incapacity, pursuant to Article 7-bis of Code 12.

3. No one may be subjected against their will to therapeutic obstinacy. Palliative care and pain relief form part of the Health Basket.

4. The right to remain at home is guaranteed, with the community support in shifts provided for by Code

12. Self-managed Common Houses are a choice of the person and may not constitute a compulsory placement.

CHAPTER III — ACCIDENTS, MEDICINES, CONFIDENTIALITY AND AUTONOMY

Art. 9 — Accidents, near misses and occupational diseases

1. Every accident, near miss or suspected occupational disease is reported and recorded without delay. The operational register contains only the information necessary for the analysis of the event, including:
   a) a description of what happened;
   b) the task being performed;
   c) the applicable Safety Sheet;
   d) the PPE and protections available;
   e) the relevant operating conditions;
   f) the date and time of the event.
2. Identifying data and health data are kept separately and with restricted access in accordance with Article 7 of Code 14 and Article 11 of this Code. The information published is aggregated and anonymised.
3. The analysis distinguishes, without automatically presuming individual responsibility:
   a) wear and tear or fortuitous event, which entail the revision of the STVs and of the organisation, without automatic individual responsibility;
   b) organisational shortcomings, which require the revision of the measures by the Circle responsible and by the Health Council;
   c) intentional conduct or other relevant individual conduct, which is dealt with in accordance with Code 4.
4. The injured person is entitled to care, to the maintenance of the Basket and, during convalescence, to the proposal of an adapted alternative CBO, when compatible with their condition.
5. The Health Councils, in federated coordination as branches of the Medicine and Health Competence Circle, carry out, at least quarterly, an aggregate analysis of accidents and near misses in order to propose the updating of the STVs of Code 2 and of the Safety Sheets.
6. The prevention cycle comprises: recording of the event, analysis of the causes, updating of the measures, revision of the STVs and prevention of future events.

Art. 10 — Health Basket, medicines, phytotherapy and experimentation

1. The Health Basket forms part of the Sufficiency Basket provided for by Code 1. It includes life-saving goods whose unavailability may give rise to a vital emergency of level 1.
2. The Confederation pursues autonomy in the production or federated procurement of non-patented essential medicines and reusable devices, in accordance with the STVs of Code 2. Critical dependencies on external suppliers are recorded in the Dependency Register provided for by Code 15.
3. **Validated phytotherapy** means a plant preparation accompanied by an STV in force that indicates at least:
   a) the species and the part used;
   b) the dosage;
   c) the indications and contraindications;
   d) the known interactions;
   e) the available evidence and its limits;

f) the methods of preparation and storage.

Traditional use, on its own, does not constitute validation. Every validated preparation is recorded in the Health STV Register.

4. Critical proprietary dependency on which life-saving medicines, essential diagnostic devices or PPE depend is prohibited. Articles 4 and 5 of Code 2 and the provisions of Code 15 apply.

5. No health experimentation may be carried out on members without free, informed, specific and revocable consent. Consent is documented in writing after comprehensible information has been provided on risks, benefits, alternatives and the right of withdrawal.

6. Experimentation requires in advance:

   a) the approval of an external Panel drawn by lot in accordance with Code 4, Article 17, composed of three persons with L3 health competence not involved in the experimentation;

   b) the opinion of the Guarantee Circle on consent and proportionality;

   c) the resolution of the General Assembly in accordance with Code 3.

7. Experimentation on minors, on persons in acute crisis within the meaning of Article 8 or on persons temporarily lacking the capacity to give consent is prohibited, except in the exceptional case in which it is aimed at achieving a direct and specific benefit for the person themselves. In that case all of the following conditions must be met cumulatively:

   a) consent given in accordance with Article 2-ter: for a minor, by the minor if he or she has reached the age of sixteen, otherwise by those who hold responsibility for his or her care; for an incapable person, by the representative designated or appointed pursuant to Article 7-bis of Code 12 or Article 2-ter, paragraph 5. The trusted person is not regarded as a representative, unless they have been formally designated for that function;

   b) if the representative is not available, application of the urgent procedure provided for by Article 7-bis of Code 12 for a temporarily incapable person, including the urgent appointment of a representative by the competent body;

   c) a written concurring opinion of two independent and uninvolved L3 persons, attesting to the direct benefit, the absence of alternatives of comparable efficacy and the proportionality of the risk;

   d) authorisation of the Guarantee Circle as regards consent and proportionality;

   e) a resolution of the General Assembly in accordance with Code 3, by qualified majority and with tracking of the positions expressed;

   f) hearing and respecting the will of the person to the extent that they are able to express it.

The absence of even one of the conditions prevents the experimentation from being started.

8. Every experimentation must provide for tracking, coverage of collective responsibilities, access to care for any consequences and the right of immediate withdrawal without prejudice.

9. Violation of paragraphs 5 to 8 constitutes a serious procedural defect and entails the application of Code 4, including the effects of invalidity and the obligations of reparation.

Art. 11 — Health confidentiality and separation of data

1. The person to whom the health data relate retains control over them. The Circle’s fiduciary System performs functions of custody and technical management, without being able to use the data for its own purposes or for incompatible purposes.

2. Access to health data is regulated as follows:

   a) ordinary access belongs to the person concerned and to the trusted person designated by them, within the limits of the consent;

   b) the Health Council may access the information necessary for health surveillance, confining itself, as a rule, to fitness findings and adaptations and not to detailed diagnoses;

c) the Guarantee Circle may access the data necessary to decide an appeal, in accordance with a tracked procedure and with restricted access;

d) in the absence of consent, access by the Health Council is permitted only when it is necessary to give effect to a collective measure validly resolved upon pursuant to Article 2-bis. Access is limited to the strictly necessary data, proportionate and temporary, is tracked and, when possible, communicated to the person concerned. Review by the Guarantee Circle within seven days is permitted.

3. The Knowledge Storehouse contains exclusively:

a) Safety Sheets free of individual health data;

b) anonymised health STVs;

c) aggregate statistics on accidents;

d) validated care protocols.

It does not contain individual health data.

4. The publication of health data identifying persons by name, of lists of conditions attributable to persons or of individual maps of frailty is prohibited. Violation constitutes a serious procedural defect in accordance with Code 4.

5. Every person has the right:

a) to obtain a copy of their own health data;

b) to request their rectification;

c) to request their erasure when they are no longer relevant or necessary;

d) to request the erasure or forgetting of past episodes, without prejudice to the collective measures documented and validly resolved upon pursuant to Article 2-bis, limited to the strictly necessary information.

6. The erasure or rectification of personal data does not entail the erasure of information that has already been rendered irreversibly anonymous and used for statistics, epidemiological analyses or the updating of the STVs. Personal data are kept distinct from anonymous statistical data and it must not be possible to reconstruct them.

Art. 12 — Transitional provisions and links

1. This Code is coordinated with:

a) Code 1, Articles 27, 29, 31 and 32;

b) Code 2, for the STVs and the validation of goods;

c) Code 3, for the urgency procedures and resolutions;

d) Code 4, Article 17, for the Review Panel and the guarantees;

e) Code 5, for L2 and L3 competences, Article 4, paragraph 2, letter c), Article 4, paragraph 5, and Article 10;

f) Code 12, for the care of frail persons;

g) Code 14, for general safety;

h) Code 15, for the Dependency Register.

2. The provisional health validation provided for by Article 10 of Code 5 applies. A person who exercises a high-risk health function on the date of entry into force may maintain a provisional validation:

a) for six months for the functions corresponding to Level 3 of Code 5;

b) for twelve months for the functions corresponding to Level 2 of Code 5.

Enrolment in the validation pathway within thirty days, enhanced supervision, compliance with the STVs and traceability of activities are required. During the provisional period the person may not validate the competences of others.

3. Within twelve months of entry into force, every Circle 1 shall activate a minimum Health Storehouse and prepare the Safety Sheets for the health and health-safety tasks classified L3 and active, in accordance with Code 5, Article 4, paragraph 2, letter c).

4. For non-health L3 tasks, Code 5 and Code 14 apply; this Code does not govern their technical validation.

5. Within twenty-four months of entry into force, a federated audit on pharmaceutical autonomy and on PPE shall be carried out, with the recording of critical dependencies in the Dependency Register of Code 15.